

Skin Puncture Test and Pathergy Phenomenon

The skin prick test is generally performed at a 45° angle, with the needle inserted 3 to 5 mm intracutaneously on the volar forearm. Erythema without infiltration is considered a negative finding. A positive pathergy reaction is characterized by the development of a papule or pustule at the site of minor trauma.

Provoked oral aphthae or genital ulcers following injection or injury—such as chorioretinitis in the corneal region after photocoagulation of the ocular fundus—are also considered manifestations of a positive pathergy phenomenon. Broader definitions include the occurrence of aneurysms at vascular anastomoses and local recurrence of ulcers after resection of affected bowel segments.

Although a positive pathergy reaction is strongly associated with Adamantiades-Behçet disease, it is not pathognomonic. It may also be observed in patients with pyoderma gangrenosum, rheumatoid arthritis, Crohn disease, and genital herpes infection.

Radiologic Findings

Scintigraphic evidence of arthritis is present in approximately 50% of patients. Cranial magnetic resonance imaging (MRI) can reveal hypodense or atrophic brain changes. Electroencephalography (EEG) showing diffuse α waves is considered an abnormal finding. Angiography is useful for detecting vascular lesions.

Differential Diagnosis

Differential Diagnosis of Adamantiades-Behçet Disease

- **Oculocutaneous/mucocutaneous syndromes**
 - Erythema multiforme exudativum and variants, including Stevens-

- Johnson syndrome
- Vogt-Koyanagi-Harada syndrome
- Reiter disease (reactive arthritis)
- Bullous autoimmune diseases: Pemphigus vulgaris, cicatricial mucous membrane pemphigoid, epidermolysis bullosa acquisita
- Viral infections (herpes, coxsackievirus, echovirus)
- Syphilis
- **Articulomucocutaneous syndromes**
 - Systemic lupus erythematosus
 - MAGIC syndrome (mouth and genital ulcers with inflamed cartilage)
 - Yersiniosis
 - Arthropathic psoriasis
- **Gastrointestinal/mucocutaneous syndromes**
 - Ulcerative colitis, Crohn disease
 - Tuberculosis
 - Bowel-associated dermatitis-arthritis syndrome
- **Aphthae**
 - Recurrent aphthous stomatitis (RAS)
 - Cyclic neutropenia
 - Herpes simplex infection
- **Genital ulcers**
 - Ulcus vulvae acutum (Lipschütz ulcer)
- Herpes simplex infection
 - Sexually transmitted infections
- **Uveitis**
 - Other forms of uveitis
- **Arthritis**
 - Ankylosing spondylitis
 - Juvenile idiopathic arthritis

- **Central nervous system manifestations**

- Multiple sclerosis
- Neuro-Sweet disease

- **Pulmonary manifestations**

- Sarcoidosis

Adapted from Altenburg A et al: Epidemiology and clinical manifestations of Adamantiades-Behçet disease in Germany—Current pathogenetic concepts and therapeutic possibilities. J Dtsch Dermatol Ges 4:49, 2006 and Rogers RS 3rd: Pseudo-Behçet's disease. Dermatol Clin 21:49, 2003.

Clinical Course and Prognosis

The clinical course of Adamantiades-Behçet disease is highly variable. Diagnosis may be delayed by several years, which can negatively impact prognosis. Mucocutaneous and joint manifestations are typically the earliest signs.

Recurrent erythema nodosum and HLA-B51 positivity are risk factors for the development of superficial thrombophlebitis and vision loss. Additional risk factors for systemic vascular involvement include superficial thrombophlebitis, ocular lesions, and male sex.

A severe disease course—characterized by blindness, meningoencephalitis, hemoptysis, intestinal perforation, and severe arthritis—occurs in approximately 10% of patients. Early, aggressive treatment of posterior uveitis significantly reduces the risk of blindness.